Contoso Community Hospital

Microsoft Copilot-Assisted Project Status Packet | Internal Medicine Clinical Documentation Pilot

Prepared: October 27, 2026 | Role: Entry-level Healthcare AI Coordinator | Classification: Synthetic training materials; no real patient data

Evidence basis: Supplied discovery input packet text, sections 1-6, including the Wednesday Planner snapshot, chair meeting notes, partial EHR dictionary, pre-generated draft and simulated exposure scenario. No direct Copilot run or live Planner access is claimed.

Decision boundary: The coordinator records, checks and routes findings; clinical accuracy, incident adjudication, governance approval and final project priorities belong to designated authorities.

Component 1 | Two framework-based Copilot prompts

Prompt 1 - Drafting pattern | Wednesday pilot check-in summary

Role: You are a project documentation assistant supporting an entry-level Healthcare AI Coordinator at Contoso Community Hospital.

Task: Draft a concise, source-grounded Wednesday pilot meeting summary and an action list for internal project review. Distinguish observations, actions and items requiring a reviewer decision.

Context - pasted source excerpt (Discovery Meeting Notes, section 3):

• Current workflow: Physicians review patient information in the EHR, document the encounter, incorporate relevant results and clinical context, review the note, and finalize/sign the documentation.

• Pain point: Documentation consumes significant physician time, particularly when information must be gathered across multiple parts of the EHR.

• AI opportunity: The team is interested in whether Copilot can assist with drafting documentation while leaving clinical review and finalization with the physician.

• Human oversight: Physicians must review generated content before it becomes part of the final clinical record.

• PHI pattern: Meeting participants discussed that examples and test materials may contain direct identifiers, clinical details, or combinations of fields that could identify a patient.

• Minimum necessary: Training and project artifacts should contain only information necessary for the task. Unnecessary identifiers should be removed or masked.

• Workflow concern: Generated text may omit context, include incorrect information, or require correction. The pilot team should document these observations rather than treating generated text as authoritative.

• Escalation concern: Potential patient-safety, privacy, or security concerns should be routed through the established clinical, privacy, compliance, or senior project escalation paths.

Constraints: Use only pasted context; do not infer meeting attendees, decisions, dates, patient details, action owners or deadlines. Use no patient identifiers or unnecessary clinical detail. Clearly label any proposed action as an open follow-up. Do not make clinical decisions, approve AI use or conclude compliance. Route clinical safety to clinical lead; privacy/security to Privacy/Compliance; governance coordination to Senior AI Program Manager/PMO.

Format: Sections: (1) meeting summary (100-150 words), (2) action list table with source evidence, owner or "not specified", date or "not specified", and route, (3) open questions.

Prompt 2 - Summarization pattern | Wednesday Planner status update

Role: You are a project status summarization assistant supporting an entry-level Healthcare AI Coordinator.

Task: Summarize the Wednesday board snapshot for the upcoming executive stand-up, keeping complete, in-progress, at-risk and not-started work distinct. Call out blockers, handoffs and missing scheduling evidence.

Context - pasted Planner board excerpt (Discovery Input, section 2):

• Pilot workflow map | Clinical Operations Analyst | Complete | Needs clinical confirmation | Route to Internal Medicine Chair

• EHR data dictionary review | EHR Team | In Progress | Some fields not yet classified | Confirm data types and PHI adjacency

• Copilot draft review | AI Project Coordinator | In Progress | Generated content needs clinician review | Document observations and open questions

• Responsible AI evidence packet | Senior AI Program Manager | In Progress | Missing evidence for selected controls | Complete checklist gap log

• Privacy/access review | Privacy/Compliance | At Risk | Potential PHI exposure scenario identified | Escalate and document incident path

• Governance committee preparation | Senior AI Program Manager | Not Started | Depends on evidence packet | Assemble review materials

Constraints: Do not invent due dates, slippage, percent complete or approval. "Complete" describes Planner task status, not clinical approval. No task is demonstrably overdue because the snapshot has no due dates. Identify required escalation without determining clinical or privacy outcomes. Do not add PHI.

Format: Executive status paragraph (80-120 words), concise work-item table and separate bullets for risks, dependencies and follow-up owners; cite exact task titles.

Component 2 | Pre-generated Copilot draft (faithful source copy)

Method: License-independent option selected: copy the supplied pre-generated draft without altering it before evaluation. Source: Discovery Input, section 5. This is a clinical documentation example, **not** a Copilot-produced project status report. The source contains no generated action list or status update; those required outputs are therefore provided below as separate coordinator-prepared, source-grounded examples and must not be misrepresented as Copilot output.

Draft context: Internal Medicine follow-up documentation example.

Generated summary: Patient reports improved symptoms since the previous visit. Continue current management and follow up as discussed.

Potential issue 1: The draft may sound clinically plausible but does not show which source information supports each statement.

Potential issue 2: The phrase "continue current management" may require clinician confirmation because the specific management plan is not represented in the draft.

Potential issue 3: A generated summary could omit relevant context from the encounter.

Required human action: A clinician must review, correct as needed, and determine whether the content is appropriate before finalization.

Coordinator action: Document observable gaps or review questions; do not independently determine clinical correctness.

Meeting summary - coordinator-prepared grounded example; not a Copilot run

The Internal Medicine check-in described an EHR workflow in which physicians gather encounter information and relevant results, compose documentation and review and sign the final note. Participants identified documentation time as a burden, especially when information is dispersed across EHR sections. The pilot is exploring Copilot-assisted drafting while retaining physician review and finalization. Participants noted risks of missing context, inaccuracies and patient-identifying details in examples or test materials. Training and project artifacts should use minimum-necessary information. Clinical safety concerns

require clinical review, while suspected exposure or security issues require escalation through the established privacy/security process. No decision authorizing a broader rollout or autonomous clinical use was documented.

Action list - coordinator-prepared grounded example

Proposed follow-up / evidence	Owner / date	Route
Confirm pilot workflow map with Internal Medicine Chair (Planner: Pilot workflow map).	Clinical Operations Analyst task owner; due date not supplied.	Clinical reviewer / Internal Medicine Chair
Complete classification and PHI adjacency questions (Planner: EHR data dictionary review).	EHR Team task owner; due date not supplied.	Privacy/Compliance as needed for sensitivity
Log source gaps in generated notes and request clinician review (Planner: Copilot draft review).	AI Project Coordinator task owner; due date not supplied.	Clinical lead for content determination
Document control evidence gaps before governance preparation (Planner: Responsible AI evidence packet).	Senior AI Program Manager task owner; due date not supplied.	Senior AI Program Manager/PMO
Document and route suspected PHI exposure (Planner: Privacy/access review).	Privacy/Compliance task owner; due date not supplied.	Privacy and Security; PMO tracking

Executive status update - coordinator-prepared grounded example

Wednesday Planner snapshot: one task is marked Complete (Pilot workflow map), but clinical confirmation remains outstanding. Three tasks are In Progress: EHR data dictionary review, Copilot draft review and Responsible AI evidence packet. Privacy/access review is At Risk due to a potential PHI exposure scenario. Governance committee preparation is Not Started and depends on the evidence packet. Priority handoffs include Internal Medicine Chair confirmation, unclassified EHR fields, clinician validation of generated content and collection of missing Responsible AI evidence. The source provides no due dates or baseline schedule, so overdue status cannot be confirmed. Privacy/security concerns are routed to Privacy/Compliance; clinical questions to the designated clinical reviewer; and governance coordination to the Senior AI Program Manager.

Component 3 | Planner status review

Snapshot source: Wednesday Planner state, section 2. Categories are explicitly checked; lack of due dates is treated as missing evidence rather than proven lateness.

Category & source task	Observed issue / evidence	Coordinator route / next record
Delayed / schedule-risk: Governance committee preparation	Not Started; depends on Responsible AI evidence packet, which is In Progress and missing control evidence. No task dates supplied: delay cannot be verified.	Flag schedule uncertainty to Senior AI Program Manager/PMO; request baseline due date and dependency milestone.
Missing owner / next-step handoff: Pilot workflow map	Task owner is Clinical Operations Analyst, but named person accountable for clinical confirmation is not specified; next step is route to Internal Medicine Chair. No board task is entirely ownerless.	Clarify sign-off/handoff owner with clinical lead and PMO; do not reassign task.
Unresolved dependency: EHR data dictionary review	In Progress; some fields not yet classified, PHI adjacency unresolved.	EHR Team tracks; route PHI classification/access questions to Privacy/Compliance.
Unresolved dependency: Copilot draft review	In Progress; generated content needs clinician review.	Record validation pending; route to clinical reviewer.

Category & source task	Observed issue / evidence	Coordinator route / next record
Unresolved dependency: Responsible AI evidence packet	In Progress; selected control evidence missing. Blocks governance preparation.	Senior AI Program Manager/PMO for evidence gap log and governance tracking.
Active blocker / risk: Privacy/access review	At Risk; potential PHI exposure scenario identified.	Immediate Privacy/Compliance and Security escalation; PMO notification; no incident adjudication.
Outstanding sign-off: Pilot workflow map	Marked Complete but needs clinical confirmation.	Clinical confirmation through Internal Medicine Chair; no implied sign-off.

Component 4 | Six-dimension Copilot draft evaluation memo

To: Senior AI Program Manager/PMO lead (tracking), with clinical and Privacy/Compliance routing as noted. **Subject:** Checklist review of simulated Copilot draft in Discovery Input section 5. **Overall disposition:** Not ready for clinical use or governance evidence as an authoritative note until designated reviewers assess the gaps. Coordinator makes no clinical or compliance determination.

1. Factual accuracy

• **F1 | High | Escalation: clinical lead / Internal Medicine Chair.** "Patient reports improved symptoms" and "follow up as discussed" contain no linked encounter source; cannot establish truth from provided packet. Request clinician source validation; do not assert the claims are false.

• **F2 | High | Escalation: clinical lead.** "Continue current management" specifies a recommendation without the underlying management plan; clinician must confirm or correct before any use.

2. PHI sensitivity

• **P1 | Low / observation | Remediation: coordinator minimum-necessary check.** The provided generated summary contains no direct patient identifier; do not assert an actual PHI breach in this draft. Do not attach underlying identifiable encounter notes to this broad packet.

• **P2 | High | Escalation: Privacy/Compliance and Security.** Separately, the section 6 screenshot scenario contains patient name, medical record number and clinical data in a broadly accessible shared folder. Track it as a distinct simulated incident, not as proof the section 5 summary disclosed PHI.

3. Task ownership

• **O1 | Medium | Remediation: coordinator; escalation for assignment: Senior AI Program Manager/PMO.** The summary includes no named physician reviewer or assigned review task. Section 5 states a clinician must review and correct, but reviewer assignment is not present. Request and record assignment; do not assign one unilaterally.

4. Deadlines and dependencies

• **D1 | Medium | Escalation: Senior AI Program Manager/PMO.** Neither draft nor supplied Planner snapshot specifies a validation due date. Copilot draft review depends on clinician validation, and governance preparation depends on the Responsible AI evidence packet. Capture dependency and request timing; never invent overdue dates.

5. Tone and audience appropriateness

• **T1 | Medium | Remediation: coordinator for communication format; clinical review remains required.** "Continue current management" sounds conclusive and lacks qualification. The encounter draft is written for clinical note review, not as an executive risk/status report. Keep the clinical draft labeled provisional and prepare a separate source-grounded leadership status.

6. Scope appropriateness

• **S1 | High | Escalation: clinical lead / Internal Medicine Chair.** A coordinator cannot endorse medication or management continuation or finalize clinical documentation. The draft must remain an unapproved example pending clinician review and sign-off.

• **S2 | Medium | Escalation: Senior AI Program Manager/PMO.** Do not infer pilot deployment readiness or governance approval from this example; unresolved control evidence and privacy risk require established review channels.

Routing summary: Coordinator may record citations/gaps, remove unnecessary PHI from authorized working copies, and route. Clinical reviewers decide accuracy and patient-safety implications. Privacy/Compliance and Security assess potential exposure. Senior AI Program Manager/PMO owns assignments, schedule and governance coordination.

Component 5 | PHI-safe inputs and incident record

Part A. Redacted prompt input 1 - meeting notes

Ready-to-paste minimum-necessary meeting excerpt: "Internal Medicine clinicians review EHR information, document encounters and sign final notes. Documentation time is a burden when information is spread across EHR sections. Copilot may assist drafting, but physicians must review generated notes before finalization. Generated content can omit context or be incorrect. Use minimum-necessary project inputs; route clinical safety issues to the clinical reviewer and privacy/security concerns to Privacy/Compliance."

Structured redaction inventory: If expanded raw notes include a patient name, replace with **[PATIENT NAME]**; medical record number with **[PATIENT ID]**; clinician personal name with **[CLINICIAN NAME]**; encounter date with **[ENCOUNTER DATE]** only when the date is needed, otherwise omit it; identifying diagnosis/medication combinations with **[CLINICAL DETAILS OMITTED]**. **No actual individual patient identifiers occur in the supplied meeting excerpt**, so no factual replacement was necessary. These are redaction rules, not fabricated source content.

Removed as not necessary for the requested summary: hypothetical identifier examples, detailed data-dictionary fields, contact/identity information, and unrelated initiative email narrative.

Part A. Redacted prompt input 2 - Planner status

Ready-to-paste minimum-necessary Planner excerpt: "Pilot workflow map: Complete; clinical confirmation pending. EHR data dictionary review: In Progress; classification incomplete. Copilot draft review: In Progress; clinician review pending. Responsible AI evidence packet: In Progress; selected controls lack evidence. Privacy/access review: At Risk; suspected PHI exposure requires Privacy/Compliance escalation. Governance committee preparation: Not Started; depends on evidence packet. Task owners are the Clinical Operations Analyst, EHR Team, AI Project Coordinator, Senior AI Program Manager, Privacy/Compliance and Senior AI Program Manager, respectively. No due dates supplied."

PHI minimization: No patient identifiers appear in the provided Planner snapshot. Omit patient examples, links to identifiable records, screenshots, granular clinical content, access-control configurations and personal contact details. Preserve task labels, status, functional owners and dependency flags needed to summarize status.

Part B. Simulated incident documentation - report and route only

Incident field	Documented entry
Incident identifier	Training scenario - ID not provided; Privacy/Compliance to assign per internal procedure.
Date / detection time	Not stated in scenario; log actual detection time on intake. Report preparation: October 27, 2026.
Reporter / role	Healthcare AI Coordinator (training role); personal name not supplied.
Organization / initiative	Contoso Community Hospital / Copilot Documentation Pilot / Internal Medicine.
Setting / project stage	Shared project folder during pilot evidence preparation; mid-pilot, pre-governance review.

Incident field	Documented entry
Observed event	Project member copied a development/testing screenshot into a shared project folder; screenshot includes patient name, medical record number and clinical information.
Exposure channel / access	Folder accessible to broader project audience than minimum necessary. Exact users, download history and outside access are unknown.
Data categories	[PATIENT NAME], [PATIENT ID / MRN], [CLINICAL INFORMATION]. No actual values copied into this record.
Affected people / scale	Number of affected patients and access recipients not established; do not assume.
Risks / downstream concerns	Potential unnecessary PHI exposure and later reuse of screenshot in project materials; regulatory characterization undetermined.
Immediate documentation / bounds	Record folder location by restricted internal reference and observed audience categories, without re-sharing the image or identifiers. Pause redistribution only if authorized. No investigation or corrective action determined here.
Primary reporting destination	Prompt escalation to designated Privacy/Compliance lead and Security incident-response channel per organizational procedure.
Secondary project notification	Notify Senior AI Program Manager / PMO lead for tracking and governance coordination; route clinical harm concerns to clinical lead if identified.
Evidence handling	Preserve only process-required evidence in approved restricted location; follow Privacy/Security handling and retention instructions.
Status / authority	Reported for review; breach finding, notification obligations, containment and resolution remain with Privacy/Compliance and Security.

Final submission and evidence integrity checklist

• Two prompts: role, task, context, constraints and format explicitly labeled; source excerpts supplied.

• Source pre-generated draft copied verbatim; extra summary/action/status items accurately labeled as coordinator examples, not Copilot executions.

• Planner review assesses schedule risk, next-step owner clarity, dependencies and active blocker with exact board tasks.

• All six evaluation dimensions documented with source reference, severity and named remediation/escalation route.

• Two reduced, PHI-safe prompt input excerpts and a report-only incident form completed.

• No fabricated patient identities, timestamps, due dates, formal clinical approvals or privacy incident outcomes.

Submission note: Source materials call this a mid-implementation pilot in the task narrative, while the initiative brief names a pre-governance implementation-readiness stage; the packet uses the brief as the authoritative stage description and makes no broader deployment claim.